

FAX TRANSMISSION COVER SHEET

Sent: 04/18/2026 14:22

From: Northern Virginia Community Clinic (NVCC)

Fax: 703-555-8821

To: Johns Hopkins Neurology Clinic Intake

Fax: 410-955-xxxx

RE: Referral – Neurology (?) / Movement vs MS vs Other

Patient: Carter, Elise M.

Reason for Referral:

34F with ongoing neurologic complaints, unclear etiology. ?demyelinating vs seizure vs functional vs migraine variant. Multiple ER visits. Persistent symptoms despite normal testing (see attached). Request evaluation.

Urgency: Routine (but patient calling frequently)

Attachments: 17 pages (may be out of order)

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PATIENT DEMOGRAPHICS

Name: Elise Marie Carter

DOB: 02/11/1992

Age: 34

Sex: F

Address: 14277 Brookfield Ln, Apt 3B, Centreville, VA 20120

Phone: 571-555-1922

Alt Phone: (none listed)

Insurance: Anthem BCBS PPO

ID: XJH22994822

Group: 88921

Emergency Contact:

Mother – Diane Carter – 571-555-8831

Marital Status: Single

Occupation: “remote admin / unclear – works from home”

Primary Language: English

Allergies: NKDA (later note mentions “sensitive to penicillin??”)

PCP: Dr. Anil Verma, MD

REFERRING PROVIDER NOTE

Date: 04/15/2026

Chief Complaint: "Everything feels off in my body"

HPI:

34-year-old female with 1+ year history of intermittent neurologic symptoms including diffuse weakness, episodic tremors, gait instability, "brain fog," and sensory complaints (numbness/tingling in variable distributions). Reports episodes of "legs giving out" without warning. Also endorses intermittent speech difficulty ("words don't come out right") and occasional visual blurring.

Symptoms are inconsistent. Some days normal function, other days unable to ambulate without assistance.

Extensive prior workup (see attached). MRI brain reportedly normal (patient says "they missed something"). Neurology previously suggested possible stress-related etiology, patient dissatisfied.

Denies loss of consciousness but has had episodes described as "spacing out." No clear postictal period.

Also reports headaches (migrainous features unclear), fatigue, poor sleep.

PMH:

Anxiety

Depression

Migraine (self-reported)

"Possible autoimmune issue?" (no documentation)

Medications: see list (inconsistent compliance)

Exam:

Vitals stable

General: anxious appearing

Neuro:

Cranial nerves grossly intact

Motor: 5/5 strength initially, later "give-way" weakness noted in bilateral lower extremities

Sensation: inconsistent responses to light touch

Gait: hesitant, wide-based at times, but able to tandem briefly with encouragement

Assessment:

Chronic neurologic symptoms, unclear etiology. Consider:

– demyelinating disease (though imaging negative)

- atypical seizure disorder
- functional overlay vs primary psychiatric
- migraine variant

Plan:

Refer to tertiary center for further evaluation
Continue current meds
Encourage PT (patient reluctant)

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PRIOR NOTE #1 – ED VISIT

Facility: Inova Fair Oaks ED

Date: 01/12/2026

Chief Complaint: “I can’t move my legs”

HPI:

Patient presents with acute onset inability to move bilateral lower extremities starting this morning. Reports similar episodes in past but “not this bad.” No trauma. No bowel/bladder incontinence.

Exam:

Initial exam shows minimal voluntary movement in legs. Reflexes 2+. Sensation patchy, non-dermatomal. During exam, patient later observed moving legs when distracted.

CT head: negative

Labs: WNL

Course:

Symptoms improved spontaneously over several hours. Able to ambulate with minimal assistance prior to discharge.

Disposition:

Discharged home

Impression:

Transient neurologic deficit – unclear etiology
?conversion vs malingering vs atypical neurologic condition

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PRIOR NOTE #2 – NEUROLOGY OUTPATIENT

Provider: Dr. K. Huang

Date: 11/03/2025

Reason for Visit: “numbness + tremor”

HPI (copy-forward appears duplicated below):

Patient reports 6 months of intermittent tremor affecting both hands, worse when observed. Also describes numbness in face and arms, migratory. Occasional difficulty speaking.

No clear triggers. Reports stress at work.

Denies seizures.

HPI (duplicate):

6 months intermittent tremor both hands, numbness face/arms, speech issues. Stress at work. No seizures.

Exam:

Mental status: normal

Motor: full strength, variable effort

Tremor: inconsistent, distractible

Sensation: decreased in non-anatomic pattern

MRI brain w/wo contrast (10/2025): normal

Assessment:

No evidence of structural neurologic disease. Symptoms may be functional.

Plan:

Reassurance

Consider psychiatry referral

Follow-up PRN

Patient response: dissatisfied, requested second opinion

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PRIOR NOTE #3 – URGENT CARE

Date: 02/20/2026

CC: dizziness, “blacking out but not passing out”

HPI:

Patient describes episodes where she feels disconnected, like “watching herself.” No collapse. Lasts minutes. Occurring several times per week.

Also reports worsening fatigue and generalized weakness.

Exam:

Vitals normal

Neuro exam limited, nonfocal

Assessment:

Dizziness – unclear

?panic attacks

?partial seizures

Plan:

Referred to neurology (again)

Advised hydration

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LAB RESULTS (partial)

CBC 01/12/2026

WBC 6.2

Hgb 13.1

Plt 242

CMP 01/12/2026

Na 139

K 4.1

Cr 0.8

AST 18

ALT 15

TSH 02/2026

2.1

Vitamin B12 02/2026

412

ANA (screen) 12/2025

Negative

ESR

12 (normal)

Lyme Ab

Negative

Random cortisol (??)

“normal” (no value listed)

IMAGING REPORTS

MRI BRAIN W/WO CONTRAST

Date: 10/28/2025

Findings:

No acute infarct, hemorrhage, or mass.
No demyelinating lesions identified.
Ventricles normal.

Impression:

Normal MRI brain

MRI C-SPINE (noncontrast)

Date: 12/2025

Findings:

Mild degenerative changes at C5-6. No cord signal abnormality.

Impression:

No evidence of demyelinating disease

EEG REPORT

Date: 02/05/2026

Indication: episodes of "spacing out"

Result:

Normal awake and drowsy EEG. No epileptiform activity.

Note: absence of abnormalities does not exclude seizure disorder

MEDICATION LIST (patient-reported, may be inaccurate)

Sertraline 100 mg daily
Topiramate 50 mg BID (patient unsure if still taking)
Propranolol 10 mg PRN "for tremor"
Ibuprofen PRN
Multivitamin
Vitamin D

Magnesium supplement
Melatonin nightly
Ondansetron PRN (from prior ED visit)
Cyclobenzaprine PRN (old prescription)

Note: patient intermittently stops meds on own

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DISCHARGE SUMMARY (abbreviated)

Facility: Reston Hospital

Date: 08/2025

Reason for Admission: weakness, inability to walk

Hospital Course:

Patient admitted for evaluation of acute bilateral leg weakness. Neurology consulted. MRI brain/spine unremarkable. Strength improved during hospitalization without intervention.

PT noted inconsistent effort.

Psych consult suggested possible somatic symptom disorder.

Discharge Diagnosis:

“Neurologic symptoms, not otherwise specified”

Disposition: home

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PHYSICAL THERAPY NOTE

Date: 09/2025

Patient demonstrates variable gait pattern. At times requires assistance, at other times ambulates independently when distracted.

Strength testing inconsistent.

Recommendation: continued therapy, though patient missed 2 follow-ups

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MISC NOTE (scanned, partially cut off)

“...patient insists something is being missed. Requests repeat MRI. Reports new symptom of right arm weakness yesterday though exam today normal...”

“...mother very concerned, states patient ‘not herself’...”

“...no objective deficits...”

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VITALS TREND (fragment)

BP range: 110–128 / 70–82

HR: 68–92

Weight: 142 lb (stable)

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PAST MEDICAL HISTORY (from intake form, unverified)

Asthma (childhood)

Concussion age 14

“autoimmune issue?”

IBS

Migraine

Anxiety

Depression

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FAMILY HISTORY

Mother – hypothyroidism

Father – HTN

No known neurologic disorders (patient later states “maybe MS in distant relative?”)

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SOCIAL HISTORY

Lives alone

Works from home

No tobacco

Occasional alcohol

No illicit drug use

Stressors: work, isolation

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HANDWRITTEN ADDENDUM (fax artifact)

“pt called again 4/16 – worsening shaking + can’t type – asking if urgent appt possible”

“??consider EMU? vs movement?”

“please eval”